

**SUPERIOR COURT OF CALIFORNIA  
COUNTY OF VENTURA**

**Tentative Ruling**

---

**2026CUPP061055: MARIA DEL SOCORRO FLORES DE LOPEZ, et al. vs DIGNITY  
HEALTH, et al.**

**08/18/2026 in Department 41**

**Demurrer by St. John's Regional Medical Center**

**SUSTAINED:** Dignity Health's Demurrer to the 1<sup>st</sup> cause of action pleaded in the Complaint.

**MOOT:** Defendant Dignity Health's Motion to Strike Portions of the Complaint considering the ruling on the demurrer.

Plaintiffs may file and serve a First Amended Complaint within 30 days of this ruling.

Notice to be provided by the moving Defendant.

Defendant Dignity Health demurs to the 1<sup>st</sup> cause of action for elder abuse/neglect for failure to state facts sufficient to state a cause of action. (Code Civ. Proc., § 430.10, subd. (e).) The cause of action against moving Defendant sounds in negligence and not in elder abuse/neglect from withholding care. Moreover, Plaintiffs failed to plead a custodial relationship.

Defendant Dignity Health moves to strike Page 11, Lines 23-25 of plaintiffs Complaint, wherein plaintiffs seek attorneys' fees and against defendant and Page 11, Line 26, of plaintiffs Complaint, wherein plaintiffs seek exemplary/punitive damages against defendants. Plaintiff failed to plead specific facts of malice, oppression, or fraud and there are no statutory or contractual authority entitling plaintiffs to attorneys' fees.

The demurrer is meritless because Plaintiffs have sufficiently pleaded with the 1<sup>st</sup> cause of action. Plaintiffs pleaded that Defendant repeatedly failed to maintain the Elder's room at a safe and lawful temperature, leaving the room unreasonably cold for days despite repeated family complaints and a known malfunctioning heating system; that the Elder developed an unstageable pressure ulcer on her coccygeal area; and that the Facility failed to monitor the Elder, failed to maintain accurate records, and failed to prevent aspiration. These deliberate and repeated failures, driven by a cost-cutting scheme of understaffing, demonstrate recklessness and a conscious disregard for the Elder's safety and rights under the Elder Abuse Act. Thus, the Court should overrule the Demurrer and deny any accompanying motion to strike.

Plaintiffs' cause of action for Elder Abuse/Neglect does not meet the heightened requirements under the Act. The claims sound in Defendant's alleged failure to provide proper medical care and not elder abuse.

Defendant Dignity Health demurs to the 1<sup>st</sup> cause of action for Elder Abuse/Neglect.

"Elder abuse claims arise under the Elder Abuse Act found in sections 15600 et seq. 'The Elder Abuse Act makes certain enhanced remedies available to a plaintiff who proves abuse of an elder, i.e., a "person residing in this state, 65 years of age or older." [Citation]. In particular, a

**2026CUPP061055: MARIA DEL SOCORRO FLORES DE LOPEZ, et al. vs DIGNITY  
HEALTH, et al.**

plaintiff who proves “by clear and convincing evidence” both that a defendant is liable for physical abuse, neglect or financial abuse (as these terms are defined in the Act) and that the defendant is guilty of “recklessness, oppression, fraud, or malice” in the commission of such abuse may recover attorney fees and costs. [Citation].” (*Worshom v. O’Connor Hospital* (2014) 226 Cal.App.4<sup>th</sup> 331, 336.)

Welfare and Institutions Code section 15610.07, subdivision (a) states:

- (a) “Abuse of an elder or a dependent adult” means any of the following:
  - (1) Physical abuse, neglect, abandonment, isolation, abduction, or other treatment resulting physical harm or pain or mental suffering.
  - (2) The deprivation by a care custodian of goods or services that are necessary to avoid physical harm or mental suffering.
  - (3) Financial abuse, as defined in Section 15610.30.

Welfare and Institutions Code section 15610.57 states:

- (a) “Neglect” means either of the following:
  - (1) Negligent failure of any person having the care or custody of an elder or a dependent adult to exercise that degree of care that a reasonable person in a like position would exercise.
  - (2) The negligent failure of an elder or dependent adult to exercise that degree of self-care that a reasonable person in a like position would exercise.
- (b) Neglect includes, but is not limited to, all the following:
  - (1) Failure to assist in personal hygiene, or in the provision of food, clothing, or shelter.
  - (2) Failure to provide medical care for physical and mental health needs. A person shall not be deemed neglected or abused for the sole reason that the person voluntarily relies on treatment by spiritual means through prayer alone in lieu of medical treatment.
  - (3) Failure to protect from health and safety hazards.
  - (4) Failure to prevent malnutrition or dehydration.
  - (5) Substantial inability or failure of an elder or dependent adult to manage their own finances.

(6) Failure of an elder or dependent adult to satisfy any of the needs specified in paragraphs (1) to (5), inclusive, for themselves because of poor cognitive functioning, mental limitation, substance abuse, or chronic poor health.

(c) Neglect includes being homeless if the elder or dependent adult is also unable to meet any of the needs specified in paragraphs (1) to (5), inclusive, of subdivision (b).

“As used in the Act, neglect refers not to the substandard performance of medical services but, rather, to the ‘failure of those responsible for attending to the basic needs and comforts of elderly or dependent adults, regardless of their professional standing, to carry out their custodial obligations.’ [Citation].” (*Covenant Care, Inc. v. Superior Court* (2004) 32 Cal.4th 771, 783.)

“[F]acts constituting the neglect and establishing the causal link between the neglect and the injury ‘must be pleaded with particularity,’ in accordance with the pleading rules governing statutory claims.” (*Carter v. Prime Healthcare Paradise Valley, LLC* (2011) 198 Cal.App.4th 396, 407.) “As to the first hospitalization, plaintiffs allege Grant was admitted for chest pains following recent hip surgery and had no pressure ulcers at that time. Nothing is alleged about the Hospital’s denial or withholding of any care or about any injury Grant suffered during this hospitalization. Thus, no violation of the Elder Abuse Act was stated based on this hospitalization. (*Ibid.*) “During the second hospitalization, plaintiffs allege that Grant was found to be malnourished and to have pneumonia, sepsis, and a pressure ulcer on his lower back and buttocks, which developed while he was at the Center; and that he developed additional pressure ulcers on his heels, which the Hospital ‘fraudulently and falsely’ documented as “there one day and then disappearing the next.’ Again, no facts are alleged as to any care or treatment the Hospital denied or withheld from Grant—indeed, the allegations that various conditions were diagnosed and that Grant was able to be discharged eight days after admission suggest the Hospital provided adequate treatment. Further, although it is alleged that during this hospitalization Grant suffered additional pressure ulcers on his heels, which were falsely documented, there are no allegations as to how the Hospital or its false documentation caused the ulcers or any other injury to Grant. Thus, no violation of the Elder Abuse Act was stated based on Grant’s second hospitalization. (*Id.* at pp. 407-408.) “As to the third and final hospital admission, plaintiffs allege that Grant died because the Hospital did not administer the antibiotics Grant needed to treat his pneumonia and did not have the proper size endotracheal tube in the crash cart, despite ‘false records’ to the contrary. Plaintiffs also allege, however, that during this hospitalization, ‘bags containing fluids [were] being injected into [Grant],’ and after ‘personnel treating [Grant] ... could not locate a common size endo-tracheal tube in the crash cart,’ they began ‘a search for an appropriate tube elsewhere in the hospital.’ These allegations indicate the Hospital did not deny services to or withhold treatment from Grant—on the contrary, the staff actively undertook to provide treatment intended to save his life.” (*Id.* at p. 408.)

Here, the Complaint alleged that Ms. De Lopez was transferred to Hospital on February 3, 2025, in severe respiratory distress, tachypneic, weak, and unable to manage her secretions. Imaging would later reveal near complete opacification and collapse of her right lung, with patchy opacities in the left lung consistent with advanced pneumonia and mucus plugging. (Compl, ¶ 12.) “Upon arrival at Hospital, Elder met clinical criteria for sepsis considering her pneumonia,

**2026CUPP061055: MARIA DEL SOCORRO FLORES DE LOPEZ, et al. vs DIGNITY HEALTH, et al.**

hypoxia, acute respiratory failure, frailty, and a recent history of abdominal infection. Despite these indicators, Hospital failed to activate a sepsis protocol.” (Compl, ¶ 13.) “Hospital also initiated BiPAP (non-invasive ventilation) despite Elder’s inability to protect her airway and inability to manage secretions. Rather than securing Elder’s airway with timely intubation, Hospital allowed her to remain on BiPAP despite her ongoing deterioration. When she was eventually intubated, bronchoscopy revealed thick purulent secretions obstructing her airways.” (Compl, ¶ 14.) “Elder remained critically ill with severe pneumonia, septic shock, and respiratory failure. Her white blood cell count rose significantly, her hemodynamics became unstable, and she required vasopressor support.” (Compl, ¶ 15.) “On February 5, 2025, despite still being septic, debilitated, and dependent on ventilatory support, Elder was prematurely extubated. St. John’s staff failed to perform adequate readiness assessments, failed to ensure Elder could protect her airway, and failed to recognize that her secretions, muscle weakness, and oxygen requirements made her a poor extubation candidate.” (Compl, ¶ 16.) “After extubation, Elder’s respiratory status deteriorated rapidly. St. John’s staff failed to provide appropriate continuous post-extubation monitoring, failed to promptly intervene, and failed to timely re-intubate or rescue her. Elder suffered an acute cardiopulmonary arrest shortly after extubation and, despite resuscitative efforts, died on February 5, 2025.” (Compl, ¶ 17.)

These allegations, like *Carter v. Prime Healthcare Paradise Valley, LLC, supra*, 198 Cal.App.4<sup>th</sup> 396, do not sufficiently plead an Elder Abuse claim, only a claim for negligence.

The Opposition cites paragraphs 6-10 of the Complaint. However, these allegations relate to Oxnard Manor, LP, identified as “Facility” under paragraph 3 of the Complaint. Reliance on paragraphs 19 and 27-30 and 34 of the Complaint is also unavailing since these are conclusory statements of duty and liability, not facts showing elder abuse or neglect.

Moreover, a claim for Elder Abuse requires “the existence of a robust caretaking or custodial relationship—that is, a relationship where a certain party has assumed a significant measure of responsibility for attending to one or more of an elder’s basic needs that an able-bodied and fully competent adult would ordinarily be capable of managing without assistance.” (*Winn v. Pioneer Medical Group, Inc.* (2016) 63 Cal.4<sup>th</sup> 148, 158.) Here, there is insufficient allegation of a caretaking and custodial relationship.

This case is unlike *Sababin v. Superior Court* (2006) 144 Cal.App.4<sup>th</sup> 81 and *Fenimore v. Regents of the University of California* (2016) 245 Cal.App.4<sup>th</sup> 1339, since there is insufficient allegation of custodial care.

Based on the foregoing, the demurrer may be sustained with leave to amend. (*Lueras v. BAC Home Loans Servicing, LP* (2013) 221 Cal.App.4<sup>th</sup> 49, 69 [leave to amend must be granted if there is a reasonable possibility the defect can be cured by amendment].)

Any party, within the time allowed to respond to a pleading, may serve and file a motion to strike the whole or any part thereof. (Code Civ. Proc., § 435, subd. (b)(1); Cal. Rules Court, rule 3.1322, subd. (b).) The Court may, upon motion, or at any time in its discretion and upon terms it deems proper: (1) strike out any irrelevant, false, or improper matter inserted in any pleading; or (2) strike out all or any part of any pleading not drawn or filed in conformity with the laws of California, a court rule, or an order of the court. (Code Civ. Proc., § 436, subds. (a) & (b);

**2026CUPP061055: MARIA DEL SOCORRO FLORES DE LOPEZ, et al. vs DIGNITY HEALTH, et al.**

*Stafford v. Shultz* (1954) 42 Cal.2d 767, 782.) When the defect which justifies striking a complaint is capable of being cured, the court should allow leave to amend. (*Perlman v. Municipal Court* (1979) 99 Cal.App.3d 568, 575.)

“A motion to strike out, or, as often termed, ‘a motion to strike,’ is traditionally used to reach certain kinds of defects in a pleading that are not subject to demurrer.” (5 Witkin, Cal. Proc. (5th ed. 2020) Plead § 1008.) “A motion to strike can also be used as a ‘scalpel’—to cut out any ‘irrelevant, false or improper’ matters inserted therein.” (Weil & Brown, Cal. Prac. Guide, Civil Proc. before Trial (The Rutter Group rev. June 2024) ¶ 7:177.) Nevertheless, “use of the motion to strike should be cautious and sparing” and should not be used as “a procedural ‘line-item veto’ for the civil defendant.” (*PH II v. Superior Court* (1995) 33 Cal.App.4th 1680, 1683.)

Defendant Dignity Health moves to strike Page 11, Lines 23-25 and Page 11, Line 26, of Plaintiffs Complaint, which seek attorney’s fees and exemplary/punitive damages against defendants.

Welfare and Institutions Code section 15657 states:

If it is proven by clear and convincing evidence, or by a preponderance of the evidence pursuant to Section 15657.02, that a defendant is liable for physical abuse as defined in Section 15610.63, neglect as defined in Section 15610.57, or abandonment as defined in Section 15610.05, and that the defendant has been guilty of recklessness, oppression, fraud, or malice in the commission of this abuse, the following shall apply, in addition to all other remedies otherwise provided by law:

(a) The court shall award to the plaintiff reasonable attorney's fees and costs. The term “costs” includes, but is not limited to, reasonable fees for the services of a conservator, if any, devoted to the litigation of a claim brought under this article.

(b) The limitations imposed by Section 377.34 of the Code of Civil Procedure on the damages recoverable shall not apply. However, the damages recovered shall not exceed the damages permitted to be recovered pursuant to subdivision (b) of Section 3333.2 of the Civil Code.

(c) The standards set forth in subdivision (b) of Section 3294 of the Civil Code regarding the imposition of punitive damages on an employer based upon the acts of an employee shall be satisfied before any damages or attorney's fees permitted under this section may be imposed against an employer.

Thus, the two claims are based on a viable claim for Elder Abuse/Neglect under the 1<sup>st</sup> Cause of Action. Since the cause of action is not sufficiently plead, the Court finds that the motion to strike is moot.

**2026CUPP061055: MARIA DEL SOCORRO FLORES DE LOPEZ, et al. vs DIGNITY  
HEALTH, et al.**

1. Motion to Strike: Find to be moot.
2. Order Plaintiff to file a First Amended Complaint.